

Global Advocacy Data Hub (GADH) statement to the Global Fund Board

The current funding context for community systems

The outcomes of the Replenishment Summit in Johannesburg in 2025 are a testament to the important role of the Global Fund and its well-earned reputation as an impactful, evidence-informed, partnership model. We thank and applaud the Secretariat, all donor countries and partners, and the work of advocates worldwide in securing these pledges.

However, the high-level success of the Replenishment should not mask the **growing risks to community systems worldwide**.

This 54th Board Meeting marks an important opportunity to safeguard the community systems that are mission critical to the Global Fund. These partners play an essential role in the public health response, generating real-time program intelligence, identifying service delivery gaps, monitoring rights violations, and designing locally appropriate prevention and treatment responses backed by GF data. Against a backdrop of reduced bilateral investments in community systems, the Global Fund has a **critical imperative to sustain community investments** as a mechanism to strengthen program quality, accountability, and responsiveness to emerging health challenges.

The implications of reduced funding on Global Fund governance and community leadership

The consultative and inclusive decision-making infrastructure that the Global Fund partnership has developed over the past two decades faces an existential risk. In response to an unprecedented lack of financial stability, we anticipate the Global Fund needing to adopt more fluid grant-making mechanisms (i.e. more frequent reprioritization, grant revisions, and portfolio optimization), which will allow the Secretariat to adjust country funding envelopes dynamically in response to the availability of resources.

In practice, **communities have only one opportunity to engage in decision-making** about Global Fund grants: during the national consultation process. [Real-world evidence](#) makes clear that grant-making, grant revisions, and portfolio optimization are effectively closed to voices outside of the PRs and Ministries of Health. Every time a grant is changed from what was agreed at the Funding Request stage, community voices are excluded from meaningfully participating in decision-making processes. In practice, this means **drifting away from Global Fund's core principles of community engagement and leadership**.

We acknowledge the efforts made by the Global Fund Secretariat to disseminate clear guidelines about reprioritization, in an effort to protect essential life-saving programs and key enablers. However, the evidence from the GC7 reprioritization exercise is clear: **the current process for revising grants does not protect community voices** or community programs.

According to [one analysis](#), 40% of community members of CCMs were unaware of any opportunity for community consultation, 55% faced challenges understanding the reprioritization process, 38% faced difficulties engaging due to lack of financial facilitation, and many described rushed timelines and tokenistic participation in which decisions had already been made by PRs and the Secretariat. Ultimately, despite Secretariat guidance to the contrary, some of the [largest proportional cuts](#) made were to human rights programs and community systems.

In GC8, the Global Fund must:

1. Produce clear guidance that **explicitly recognizes community priorities as life-saving** interventions and engage with the PRs and CCMs to ensure that these activities are protected throughout GC8 implementation.
2. **Increase CCM budget lines** to support engagement in each reprioritization exercise, with clear minimum requirements for community consultation costs.
3. Ensure that all **guidance is directly and widely shared** with all CCM representatives. Dissemination must include investments in tools that translate technical policy, epidemiological, and funding data into formats that are culturally relevant, linguistically accessible, and usable by grassroots advocates and community networks. Investments should also support regional and cross-country knowledge exchange platforms that enable community advocates to share lessons, strategies, and advocacy evidence, strengthening coordinated engagement in Global Fund processes.
4. Global Fund guidance must enforce actionable reporting on community engagement metrics. This must include **mandating formal CCM sign-off and proof of consultation** for all phases of the grant cycle, including grant-making, grant oversight, and grant revision activities.

Integration, transition, and protecting community funding streams

We are deeply appreciative of the Secretariat's efforts to protect community funding streams in GC8, including through a community-focused funding stream, a renewed focus on social contracting, and leveraging multi-country grants. These approaches will be essential for responding to emerging needs and resourcing community activities in recently-transitioned countries.

In principle, the efforts to integrate donor-supported programs into national systems is an essential step toward long-term sustainability and country ownership. [Communities acknowledge](#) that **integration is necessary for long-term sustainability** and stronger health systems, with the involvement of communities. They recognise that a well-integrated primary healthcare (PHC) system can streamline care, reduce parallel structures, improve chronic disease management, and offer holistic services mental health, TB screening, malaria testing, SRHR that are often fragmented today.

However, communities emphasise that **the current PHC landscape in many African countries is not ready for integration**, especially for KPs who face compounded vulnerabilities due to criminalisation, stigma, gender identity biases, and unsafe public environments. Without guardrails, integration risks transferring clients from safe, community-led

spaces into hostile PHC settings that can expose them to discrimination, embarrassment, violence, or even arrest. This tension between aspiration and risk defines the integration moment for Africa.

Early evidence from integration efforts highlight a poorly-defined process, weak guardrails, discriminatory practices, and the shuttering of trusted spaces for service delivery.

Community-led services are country-owned and a core pillar of the national response to the three diseases, which are a response to decades of implementation science and real-world evidence. Community-led advocacy and engagement models play a critical role in maintaining trust and service uptake during integration and transition processes. The Global Fund should recognize and invest in innovative, community-driven communication and accountability approaches that strengthen engagement with vulnerable populations and ensure continuity of services.

As such, we highlight three axioms that will act as fundamental barriers to integration and transition:

- **Social contracting is not possible in countries that criminalize** key and vulnerable populations. With narrow exceptions, in such countries there is no possibility of protecting community-led programming contracted by Ministries of Health. Nearly every Global Fund country has policies in place criminalizing one or more key and vulnerable population groups.
- **Government PRs rarely subcontract to community organizations.** Recent [evidence suggests](#) that in most Global Fund-supported countries, the only way that community-led sub-recipients or sub-sub-recipients are funded is through civil society PRs.
- **Contracts with other bilateral funders may block government and UN PRs from implementing certain community programs.** In these scenarios, regardless of the application of these restrictions to the Global Fund itself, bilateral agreements made with other donors create a significant barrier to these PRs implementing community-focused programs.

As such, in GC8 the Global Fund must:

1. **Require dual-track financing in every country** that does not have established and operational social contracting mechanisms that are accessible to community-led organizations. These pathways must be clearly institutionalized and documented before a shift toward a single government PR may be considered.
2. **Develop TRP guidance that triggers in-depth review** in all cases where community and KVP-focused programs are removed from Global Fund grants and integrated into government systems.
3. **Clearly define integration and minimum criteria.** Misconceptions about integration are currently acting as a barrier to thoughtful transfer of programs to country systems. Global Fund guidance must be very explicit about minimum criteria and expectations, including demonstrating sufficient domestic funding and appropriate timelines to retrain healthcare workers, hire sufficient staff, and develop community-based infrastructure. Integration should not be promoted as a rapid response nor as a cost-saving measure. Integration

planning should include safeguards for sustaining community advocacy ecosystems, including investments in community capacity strengthening, advocacy coordination platforms, and leadership development for emerging community advocates, particularly youth-led structures.

4. **Protect Global Fund funding streams that advance transition and integration readiness**, including for monitoring domestic resources and changing laws and policies. These program areas have been disproportionately impacted during the reprioritization exercise and must be robustly resourced in GC8.

Catalytic Investments for GC8

Catalytic investments (CIs) are vital to drive affordability and safeguard community engagement by strengthening community accountability systems, enabling community-generated evidence for advocacy, and supporting policy dialogues in national and global health decision-making spaces. These investments become even more vital during periods of funding reductions where relatively small allocations to CIs can generate large global gains in affordable commodity procurement and continued community engagement.

[Analyses](#) of GC7 reprioritization found that **community and gender programs were at high-risk for deallocation**, despite Global Fund guidance to maintain these allocations. Interventions with the largest percent reduction in funding during reprioritization included removing human rights and gender-related barriers to vector control programs (77% reduction), community-led advocacy and monitoring of domestic resource mobilization (71%), monitoring and reforming policies, regulations and laws (69%), addressing needs of people in prisons and other closed settings (61%), collaboration with other programs/sectors (54%), and community-based continuous distribution of insecticide treated nets (54%).

Increasing allocations for the CI for Community Network and Engagement and Human Rights and Gender is a key strategy to protect community priorities during potential additional reprioritization efforts and will pay dividends towards better functioning Global Fund programming. We do not find this request to be in opposition to the Country Allocation process, but rather in direct support of country work. Increased funding for community and equity focused CIs could be through a concerted effort to increase collaborations with private donors seeking to support community engagement and programs.

In GC8, the Global Fund should increase CI funding levels for Community Network and Engagement and Human Rights and Gender.